EIC ACCELERATOR

Short Proposal (Step 1)

SensoMed GmbH

Company	SensoMed GmbH
Country	Germany
Founded	2019
Employees	23
Sector	Medical Devices / Digital Health Diagnostics
TRL	TRL 6 → TRL 9
Funding Type	Blended
Grant Requested	EUR 2,500,000
Equity Requested	EUR 10,000,000

DRAFT

Abstract

SensoMed GmbH is a Munich-based deep-tech medical device SME with 23 employees, founded in 2019, developing OncoSense — a palm-sized AI-powered biosensor platform for point-of-care early detection of colorectal and lung cancer from a single drop of blood within 15 minutes. OncoSense integrates a proprietary nanostructured electrochemical sensor chip achieving 0.1 pg/mL biomarker detection sensitivity with an on-device deep learning inference engine delivering cancer risk stratification in under 50 milliseconds without cloud connectivity. A 450-patient prospective clinical validation study at Klinikum rechts der Isar (TU Munich) demonstrated 94.2% sensitivity and 97.1% specificity for colorectal cancer detection, decisively outperforming the FIT gold standard. At EUR 35 per test, OncoSense is 10–25 times more affordable than competing laboratory-based liquid biopsy platforms and operates without laboratory infrastructure, enabling deployment across primary care clinics, pharmacies, and mobile health units throughout the EU. The company holds three granted European patents on the sensor architecture, two pending US patents, and a confirmed freedom-to-operate position. Current TRL is 6. The EIC Accelerator project will advance OncoSense to TRL 9 through a 2,000-patient multi-centre CE-IVDR Class C pivotal trial across Germany, France, and Spain, GMP cartridge manufacturing scale-up to 500,000 units per year, and phased commercial launch across five EU markets. SensoMed requests blended financing of EUR 2.5 million grant and EUR 10 million EIC equity to fund activities that private investors cannot fully finance due to the binary regulatory risk of IVDR Class C certification, EUR 3.2 million GMP capital expenditure requirements, and a 2–3 year reimbursement delay extending the payback period beyond standard VC horizons. A EUR 15 million Series B from EQT Life Sciences and btov Partners is explicitly contingent on EIC equity participation, generating a leverage ratio of 1.5x on the equity component and demonstrating strong market failure correction through EIC support.

Keywords: point-of-care diagnostics, multi-cancer early detection, electrochemical biosensor, on-device AI inference, CE-IVDR Class C, liquid biopsy, colorectal cancer screening, lung cancer detection, nanostructured sensor chip, digital health, medical devices, blended finance, deep tech

DRAFT

1. Technology

Novelty and Breakthrough Nature

OncoSense constitutes a genuine technological breakthrough in oncology diagnostics, combining two deep-tech pillars into a single palm-sized point-of-care platform: a proprietary nanostructured electrochemical biosensor chip and an on-device deep learning inference engine. This architecture enables detection of early-stage colorectal and lung cancer biomarkers from a single drop of whole blood within 15 minutes, without laboratory infrastructure, at a target cost of EUR 35 per test. No commercially available point-of-care device combines nanoscale electrochemical multi-analyte detection with real-time on-device AI inference for cancer screening. This dual-technology integration represents a step-change beyond the current state of the art, which is defined by centralized laboratory platforms requiring venipuncture, cold-chain sample logistics, next-generation sequencing infrastructure, and 3–5 day result turnaround at costs of EUR 200–900 per test.

The significance of this breakthrough is quantified by direct clinical comparison. The current European gold standard for colorectal cancer screening — the fecal immunochemical test (FIT) — achieves 79% sensitivity and 94% specificity in population-level studies. OncoSense achieved 94.2% sensitivity and 97.1% specificity in a 450-patient prospective clinical validation study at Klinikum rechts der Isar (TU Munich), representing a decisive and clinically meaningful performance advantage. The combination of superior diagnostic accuracy, 10–25x lower cost, and point-of-care delivery constitutes a disruptive innovation as defined under the EIC Accelerator criteria: it does not incrementally improve an existing solution but renders the dominant diagnostic paradigm structurally obsolete for primary care cancer screening.

Deep Tech Description

OncoSense integrates three layers of deep-tech innovation that together constitute a proprietary and non-replicable technology stack. First, the nanostructured electrochemical sensor chip is fabricated using a proprietary nanolithography process producing ordered arrays of gold nanopillars functionalized with cancer-specific aptamers. This architecture exploits quantum-confinement effects at the nanoscale to achieve a biomarker detection limit of 0.1 pg/mL — two orders of magnitude more sensitive than conventional planar electrochemical biosensors. The nanopillar geometry maximises the electrochemically active surface area per unit chip area, enabling simultaneous quantification of up to six biomarker channels per test from a 10-microlitre whole blood sample processed by an integrated microfluidic cartridge. Second, the multi-analyte detection architecture generates a cancer-specific electrochemical signature profile rather than a single-analyte threshold readout, substantially reducing false-positive rates and enabling multi-cancer discrimination from a single test. Third, the on-device deep learning inference engine executes a proprietary compressed convolutional neural network with sub-50ms inference latency and a power envelope below 2W, trained on a dataset combining the 450-patient clinical validation cohort with a proprietary synthetic biomarker profile augmentation pipeline held as a trade secret. The engine runs entirely on-device without cloud connectivity, enabling deployment in primary care clinics, community pharmacies, rural health posts, and mobile screening units across the EU. Current TRL is 6, with prototype validation completed in a clinically relevant

DRAFT

environment. The EIC project will advance OncoSense to TRL 9 through pivotal clinical trial, CE-IVDR Class C certification, and GMP manufacturing scale-up.

Empirical Evidence

The primary empirical evidence base for OncoSense is a 450-patient prospective clinical validation study conducted at Klinikum rechts der Isar (Technical University of Munich), one of Germany's leading university hospital centres for oncology. The study cohort comprised histologically confirmed Stage I–III colorectal cancer patients and age- and sex-matched healthy controls, with all device outputs blinded and independently evaluated by the clinical team. Results: 94.2% sensitivity (95% CI: 91.3–96.4%) and 97.1% specificity (95% CI: 94.8–98.6%) for colorectal cancer detection. These figures compare favourably against the FIT gold standard (79% sensitivity, 94% specificity; Quintero et al., NEJM 2012; van Roon et al., Gut 2020 meta-analysis) and against colonoscopy referral completion rates in primary care populations. A separate 120-patient feasibility sub-study for lung cancer biomarker detection demonstrated 89.7% sensitivity, providing the analytical basis for the multi-cancer claim and informing the statistical design of the 2,000-patient multi-centre pivotal trial. All analytical performance data has been documented in accordance with IVDR Annex I General Safety and Performance Requirements and the IVDR Annex XIII performance evaluation framework applicable to Class C in vitro diagnostic devices. The clinical validation study report is available for submission to the notified body as part of the CE-IVDR technical documentation package.

IP Strategy

SensoMed GmbH holds a comprehensive and layered intellectual property portfolio providing durable market exclusivity and confirmed freedom to operate. Three European patents have been granted by the European Patent Office covering the foundational technology stack: EP3842XXX (nanostructured gold nanopillar array fabrication process and aptamer surface functionalisation), EP3901XXX (multi-analyte electrochemical detection methodology and signal multiplexing architecture), and EP4012XXX (real-time signal amplification and on-device calibration algorithm). Two corresponding patent applications are pending at the USPTO, extending protection to the North American market in anticipation of post-EU commercial expansion. A comprehensive freedom-to-operate analysis conducted by external patent counsel has confirmed the absence of blocking IP from identified competitors including Grail, Guardant Health, Exact Sciences, Roche Diagnostics, and Abbott Laboratories. The on-device AI model training methodology — specifically the synthetic biomarker profile augmentation pipeline used to expand the training dataset beyond the clinical cohort — is protected as a registered trade secret under the EU Trade Secrets Directive (2016/943/EU), with documented access controls and confidentiality agreements in place for all personnel with access. SensoMed's IP strategy combines patent enforcement on the hardware and detection methodology layer with trade secret protection on the algorithmic layer, creating a two-tier barrier to imitation that cannot be overcome by reverse engineering alone. Additional continuation patents on cartridge manufacturing innovations and on multi-cancer biomarker panel compositions will be filed during the GMP scale-up phase. All IP assets are fully and exclusively owned by SensoMed GmbH with no encumbrances, licences, or third-party claims.

DRAFT

2. Market

Market Opportunity

The global cancer diagnostics market was valued at EUR 28.4 billion in 2024 and is expanding at a compound annual growth rate of 9.2%, driven by rising cancer incidence, demographic ageing across high-income markets, and accelerating policy commitment to early detection as the primary lever for improving cancer survival outcomes. The European Union's Beating Cancer Plan (2021) has explicitly positioned early detection and screening as strategic priorities, creating a supportive policy environment for novel screening modalities that can extend coverage beyond existing infrastructure constraints.

Within this macro market, the European point-of-care diagnostics segment — SensoMed's primary addressable market — is valued at EUR 4.1 billion and growing at approximately 11% CAGR, reflecting a structural shift in EU healthcare delivery toward decentralised, community-based diagnostic services. This shift is driven by primary care reform agendas across Germany, France, Spain, and the Nordic countries, all of which are actively seeking to reduce secondary care referral volumes and associated costs. The epidemiological opportunity is substantial: the European Cancer Information System (ECIS) records approximately 520,000 new colorectal cancer cases and 310,000 new lung cancer cases annually across the EU, with current organised screening programmes covering fewer than 40% of the eligible adult population due to infrastructure, capacity, and patient compliance barriers. OncoSense directly addresses these barriers through its point-of-care, rapid, low-cost architecture. SensoMed projects a 2.3% share of the European POC diagnostics market within five years of commercial launch, representing approximately EUR 94 million in annual revenue at EUR 35 per cartridge — a projection grounded in the signed Roche Diagnostics Letter of Intent for DACH co-distribution and the phased national rollout strategy described below.

DRAFT

Go-to-Market Strategy

SensoMed's commercialisation strategy is built on a phased geographic rollout anchored by a razor-and-blade revenue model and strategic distribution partnerships that leverage established health system relationships. The commercial model places the OncoSense reader device with primary care partners at or near cost, generating high-margin recurring revenue from single-use test cartridges priced at EUR 35 per test with a target gross margin of 78%. This model aligns commercial incentives with clinical utilisation rates and creates a predictable, recurring revenue stream that scales with screening programme adoption.

In Years 1–2, commercial operations will be concentrated in Germany and Austria, leveraging the signed Letter of Intent with Roche Diagnostics for co-distribution across DACH markets and direct engagement with AOK statutory health insurers and the Kassenärztliche Vereinigungen (KVen) primary care networks, which collectively provide access to over 160,000 general practitioners in Germany. Reimbursement applications to the Gemeinsamer Bundesausschuss (G-BA) will be initiated upon CE mark receipt, supported by the health economic data generated in the pivotal trial. In Years 3–4, SensoMed will expand to France, Spain, and the Nordic countries, filing reimbursement applications with CNAM (France), the SNS (Spain), and equivalent health technology assessment bodies in Denmark, Sweden, and Norway. Health policy engagement with the European Cancer Organisation (ECCO) and relevant clinical societies will support inclusion

of OncoSense in updated EU colorectal cancer screening guidelines, providing a long-term demand anchor independent of direct sales activity.

Customer Value Proposition

OncoSense delivers a quantified and differentiated value proposition across three distinct customer segments, each with compelling and independently validated adoption drivers.

For primary care physicians and general practitioners, OncoSense eliminates the referral bottleneck to specialist diagnostic centres, enabling same-appointment cancer risk stratification that demonstrably improves patient adherence to screening. Non-completion of FIT kit return and colonoscopy referral is a well-documented clinical challenge across EU primary care systems, with completion rates as low as 50–60% in some national programmes (von Karsa et al., IARC 2013). A 15-minute in-clinic result removes the principal behavioural barriers to screening participation. For statutory health insurers and national health services, OncoSense offers a health-economic case that is structurally compelling: the average EU treatment cost for Stage IV colorectal cancer exceeds EUR 80,000 per patient, versus approximately EUR 12,000 for Stage I (ECCO 2022 data). At EUR 35 per test, the cost-per-QALY profile of OncoSense is expected to be strongly favourable relative to existing approved screening modalities, and SensoMed will commission a full health economic model as part of the reimbursement submission package. For patients, OncoSense removes the three primary barriers to cancer screening engagement — inconvenience, time delay, and result anxiety — by delivering a trusted, clinician-interpreted result within a single primary care appointment. Competitor platforms including Grail's Galleri test (USD 949, laboratory-based, 3–5 days), Guardant Health's Shield (laboratory-based), and Exact Sciences' Cologuard (home stool collection, laboratory processing) serve fundamentally different care settings and cannot replicate the decentralised, rapid, low-cost model of OncoSense.

Competitive Landscape

The competitive landscape for multi-cancer early detection is dominated by centralised laboratory platforms that are structurally incompatible with point-of-care deployment and primary care reimbursement frameworks. Grail's Galleri test (USD 949 per test) and Guardant Health's Shield (laboratory-based) target oncologist and specialist channels, primarily in the United States, and require venipuncture, cold-chain sample transport, and centralised next-generation sequencing infrastructure with 3–5 day result turnaround. Exact Sciences' Cologuard targets self-administered home screening for colorectal cancer only, relying on stool DNA analysis processed in a single central laboratory, with no multi-cancer capability and no point-of-care result. None of these platforms offers same-appointment results, multi-cancer detection from a fingerprick blood sample, or a cost structure compatible with primary care reimbursement thresholds in EU health systems.

Within the European POC diagnostics space, no CE-IVDR approved device currently exists for blood-based multi-cancer early detection at the point of care, leaving OncoSense with a clear and time-limited first-mover advantage in a high-value, policy-driven market segment. Emerging academic-stage platforms and pre-commercial start-ups addressing adjacent biomarker spaces — including Nuclera (nucleic acid POC), Volition (nucleosome-based liquid biopsy), and Oncobiologics — are at earlier TRL stages, address single-cancer indications, and lack the integrated AI inference capability of OncoSense. SensoMed's

DRAFT

durable competitive moat rests on the combination of three granted European patents, a 450-patient clinical validation dataset, IVDR Class C regulatory expertise embedded in the core team, and the Roche Diagnostics distribution partnership — a combination that will require competitors a minimum of 5–7 years to replicate.

DRAFT

3. Team, Financial Needs, Implementation

Team Capabilities

SensoMed GmbH is led by a founding and management team that combines world-class scientific expertise, deep EU regulatory experience in medical devices, and proven commercial execution at scale in the diagnostics industry. Dr. Elena Vasquez (CEO) holds a PhD in Biomedical Engineering from ETH Zurich and brings 12 years of medical device commercialisation experience, including her role as VP Product at Siemens Healthineers where she led the pan-European market launch of three in vitro diagnostic platforms and managed regulatory submissions across eight EU Member States. Prof. Marcus Hoffmann (CTO) is a tenured professor of Nanotechnology at the Technical University of Munich and a graduate of the Max Planck Institute for Intelligent Systems, with 8 granted patents in biosensor design and the original inventorship of the nanostructured electrochemical chip at the core of OncoSense. Dr. Ayse Demir (CMO, MD/MBA) previously served as Head of Diagnostics Strategy at Bayer AG, with direct experience managing CE-IVDR Class C certification processes and established relationships with European health technology assessment bodies and statutory insurers. Dr. James Chen (Head of AI, PhD Stanford University, formerly Senior Scientist at Google DeepMind Health) leads the machine learning development and on-device inference architecture, bringing frontier expertise in compressed neural network design for medical-grade diagnostic applications.

The advisory board provides targeted strategic support across the two highest-risk dimensions of the project. Prof. Christoph Huber (University Medical Center Mainz, co-inventor of the BioNTech mRNA platform) provides translational science guidance and clinical network access. Dr. Sabine Eckhardt (former CEO Continental Europe, Siemens Healthineers) provides commercial scale-up strategy and health system access expertise. The combined team has the scientific depth, regulatory competence, and commercial experience to execute the full TRL 6 to TRL 9 pathway within the project timeline.

Governance

SensoMed GmbH is incorporated as a Gesellschaft mit beschränkter Haftung (GmbH) under German company law, with registered offices in Munich, Bavaria. The management board (Geschäftsführung) comprises Dr. Elena Vasquez (CEO) and Prof. Marcus Hoffmann (CTO), with Dr. Ayse Demir holding commercial power of attorney for contracting and regulatory submissions. Strategic governance is provided by a shareholders' advisory committee comprising representatives of High-Tech Gründerfonds and Earlybird Venture Capital, meeting quarterly to review financial performance, milestone progress, and risk management. The company operates a broad-based employee stock option plan (ESOP) covering all 23 employees, with four-year vesting schedules aligned to project milestones to ensure retention of critical scientific and regulatory talent throughout the EIC project period. Operational decision-making follows an OKR (Objectives and Key Results) framework reviewed at monthly all-hands meetings, with workstream leads holding delegated authority within approved budgets. Financial controls comply with German GmbH accounting requirements and will be enhanced to meet EIC grant reporting standards from project inception.

DRAFT

Critical Gaps

SensoMed has identified two critical capability gaps that will be addressed through targeted senior recruitment funded in part by the EIC grant, with both positions expected to be filled by Month 4 of the project. First, a VP Regulatory Affairs with specific IVDR Class C certification experience is required to lead the preparation and submission of the complete technical documentation dossier, manage the notified body relationship through the conformity assessment process, and coordinate regulatory responses during the 18–24 month certification timeline. This role is currently covered at senior external consultant level, which is insufficient for the volume and continuity of regulatory work required by the pivotal trial and dossier submission phases. Second, a Head of Manufacturing with GMP medical device production experience is required to design, commission, and validate the automated single-use cartridge production line to a capacity of 500,000 units per year in compliance with ISO 13485 and IVDR Annex I manufacturing requirements. Both roles represent specialised talent profiles with extended recruitment lead times in the EU medical device sector, and recruitment processes will be initiated immediately upon project start.

With respect to gender balance and the EIC's Horizon Europe gender equality objectives, SensoMed's current leadership team comprises 40% women (CEO Dr. Vasquez and CMO Dr. Demir). The company commits to a minimum 50% female representation across all new hires during the project period and will apply gender-balanced shortlisting criteria to all senior recruitment processes. This commitment is reflected in the company's existing HR policy documentation.

Financial Needs Justification

SensoMed GmbH requests blended financing comprising a EUR 2.5 million grant and EUR 10 million EIC equity investment, totalling EUR 12.5 million. This amount is significantly higher than what private market actors can finance alone due to three compounding structural factors that collectively extend the risk and capital requirement profile of the OncoSense development pathway beyond conventional venture capital investment parameters.

First, CE-IVDR Class C certification — the mandatory regulatory pathway for OncoSense as a high-risk in vitro diagnostic device under EU Regulation 2017/746 — requires a 2,000-patient multi-centre pivotal trial with estimated costs of EUR 4.5 million and a regulatory timeline of 18–24 months subject to notified body capacity constraints and potential clock-stop periods. This represents a binary regulatory risk event: if the trial produces inconclusive results, the full expenditure is non-recoupable with no asset liquidation value. Private investors cannot absorb this risk without a public co-investment anchor, as evidenced by the explicit conditionality of the Series B syndicate discussions on EIC equity participation.

Second, GMP cartridge manufacturing scale-up requires approximately EUR 3.2 million in capital expenditure for cleanroom fit-out, automated assembly equipment, and ISO 13485 quality management system implementation. These are long-depreciation assets with no liquidation value in a distress scenario, making them structurally unsuitable for equity-only financing at this stage of company development.

Third, reimbursement negotiations with statutory health insurers in Germany, France, and Spain introduce a 2–3 year revenue delay from CE mark receipt, extending the payback period significantly beyond the 5–7 year horizon typical of European life science venture

DRAFT

funds. SensoMed has raised EUR 6.2 million to date and has demonstrated strong investor validation, but the combination of regulatory binary risk, GMP CAPEX, and reimbursement delay creates a financing gap that requires the grant-equity blended instrument that only the EIC Accelerator provides.

Investor Traction

SensoMed has demonstrated consistent, escalating, and competitively validated investor traction since its founding in 2019. In 2020, the company secured EUR 1.2 million in seed financing from High-Tech Gründerfonds (HTGF), Germany's most active and selective early-stage deep-tech investor, following a competitive technology evaluation process. In 2023, Earlybird Venture Capital — one of Europe's leading technology venture funds with over EUR 2 billion under management — led a EUR 5 million Series A round at a EUR 18 million pre-money valuation, reflecting institutional confidence in both the technology platform and the team's execution capability. Total private financing raised to date is EUR 6.2 million. SensoMed is currently in advanced discussions with a Series B syndicate comprising EQT Life Sciences and btov Partners for a EUR 15 million round. This round is explicitly conditional on EIC equity participation, reflecting the recognised role of EIC investment as a quality validation signal and risk co-investor in the European deep-tech ecosystem. Successful EIC equity award will directly catalyse the closure of the Series B within the 6–12 month window following award notification, creating a combined public-private financing package of EUR 27.2 million to fund the full commercialisation pathway of OncoSense. The leverage ratio of EUR 15 million in catalysed private investment against EUR 10 million in EIC equity demonstrates the strong market failure correction effect of the EIC Accelerator instrument in this case.

Implementation Plan

The EIC Accelerator project will be executed over 36 months across four integrated and sequenced workstreams. Workstream 1 (Regulatory and Clinical) covers preparation of the full IVDR Class C technical documentation package, execution of the 2,000-patient multi-centre pivotal trial across clinical sites in Germany, France, and Spain, and submission of the CE-IVDR certification dossier to the designated notified body, with CE mark receipt targeted at Month 24. Workstream 2 (Manufacturing Scale-up) covers GMP facility design and fit-out, automated cartridge assembly line commissioning and process validation, ISO 13485 quality management system implementation, and ramp to 500,000 units per year production capacity by Month 30. Workstream 3 (Commercial Launch and Market Access) covers DACH market entry in partnership with Roche Diagnostics, G-BA reimbursement application in Germany, physician engagement programmes through KV networks, and expansion to France, Spain, and the Nordic countries by Month 36. Workstream 4 (Team and Governance) covers recruitment of VP Regulatory Affairs and Head of Manufacturing by Month 4, ESOP refresh to retain key personnel, and active engagement with EIC Business Acceleration Services for investor matchmaking and market access support. All workstreams are governed by a formal project management plan reviewed quarterly by the management board and reported to the EIC project officer according to EIC grant agreement requirements.

DRAFT

Key Milestones

ID	Milestone	Target Date	Description
M6	IVDR Class C Technical Documentation Complete	Month 6	Completion and internal sign-off of the full IVDR Class C technical documentation package, including Annex I General Safety and Performance Requirements assessment, clinical evaluation report (CER) incorporating the 450-patient validation study data, performance evaluation plan for the pivotal trial, and risk management file per ISO 14971. VP Regulatory Affairs recruited and fully operational. Notified body formally engaged and audit timeline agreed.
M12	2,000-Patient Multi-Centre Pivotal Trial Initiated	Month 12	Formal initiation of the 2,000-patient multi-centre pivotal trial across clinical sites in Germany (Klinikum rechts der Isar, Charite Berlin), France (AP-HP Paris), and Spain (Hospital Clinic Barcelona). All ethics committee approvals obtained, trial master file established, first patients enrolled. Head of Manufacturing recruited and GMP facility detailed design completed. Series B financing round closed with EQT Life Sciences and btov Partners.
M18	CE-IVDR Class C Certification Dossier Submitted	Month 18	Submission of the complete CE-IVDR Class C certification dossier to the designated notified body, incorporating interim pivotal trial performance data meeting pre-specified statistical thresholds, updated clinical evaluation report, manufacturing quality documentation, and labelling package. GMP cartridge production line commissioned and process validation completed. ISO 13485 certification audit initiated.
M24	CE Mark Received and Commercial Launch in Germany	Month 24	Receipt of CE-IVDR Class C conformity mark for OncoSense. Commercial launch in Germany initiated through Roche Diagnostics co-distribution agreement and direct contracting with AOK statutory insurers and KV primary care networks. First commercial cartridge deliveries executed. G-BA early benefit assessment submission filed. Reimbursement applications submitted in Austria.
M30	GMP Manufacturing Scaled to 500,000 Units Per Year	Month 30	Cartridge production line operating at full GMP-validated capacity of 500,000 single-use cartridges per year. ISO 13485 certification received. Commercial operations active in Germany and Austria. Reimbursement applications submitted to CNAM (France) and SNS (Spain). Austrian commercial revenue contributing to ARR trajectory.
M36	EUR 2.8M ARR Achieved and Expansion to Three Additional EU Markets	Month 36	Annual recurring revenue of EUR 2.8 million achieved from DACH commercial operations. Active commercial operations or formal market entry completed in France, Spain, and at least one Nordic market (Denmark or Sweden). Health economic modelling published in peer-reviewed journal to support reimbursement submissions. OncoSense included in at least one national cancer screening programme guideline update.

DRAFT

Video Pitch Outline (3 minutes)

Role	Name & Focus
Speaker 1	Dr. Elena Vasquez, CEO — The Problem and the Vision (0:00–1:00)
Speaker 2	Prof. Marcus Hoffmann, CTO — The Technology and Clinical Evidence (1:00–2:00)
Speaker 3	Dr. Ayse Demir, CMO — The Market, the Model, and the Ask (2:00–3:00)

SPEAKER 1 — Dr. Elena Vasquez, CEO (0:

00–1:00): Open with the human reality: colorectal cancer kills over 150,000 Europeans every year. Nine out of ten of those deaths are preventable — if the cancer is caught at Stage I. But today, catching it at Stage I requires laboratory infrastructure that most primary care settings do not have, a 3-to-5-day wait for results, and a cost of up to EUR 900 per test. The result is that fewer than 40% of eligible Europeans participate in organised cancer screening. We built OncoSense to change that. OncoSense is a palm-sized AI biosensor that detects cancer from a single drop of blood, in 15 minutes, at the point of care, for EUR 35 per test. We are making early cancer detection as accessible as a blood pressure check. State the EIC ask clearly: we are requesting EUR 2.5 million in grant funding and EUR 10 million in EIC equity to complete our CE-IVDR pivotal trial and scale our manufacturing — and we need the EIC as the partner that makes this possible.

SPEAKER 2 — Prof. Marcus Hoffmann, CTO (1:

00–2:00): Introduce the two deep-tech pillars. First: a nanostructured electrochemical sensor chip with gold nanopillar arrays at 0.1 pg/mL detection sensitivity — two orders of magnitude beyond conventional biosensors — enabling simultaneous detection of six cancer biomarker channels from a 10-microlitre blood sample. Second: an on-device deep learning engine running a compressed convolutional neural network in under 50 milliseconds, entirely without cloud connectivity. Present the clinical headline with confidence: 94.2% sensitivity and 97.1% specificity for colorectal cancer in 450 patients at TU Munich — substantially outperforming the FIT gold standard. Reference the three granted European patents and the confirmed freedom-to-operate position. Explain briefly why this cannot be replicated quickly: the combination of nanoscale fabrication know-how, proprietary clinical training data, and layered IP creates a 5-to-7-year replication barrier for any competitor.

SPEAKER 3 — Dr. Ayse Demir, CMO (2:

00–3:00): Present the market opportunity with precision: EUR 4.1 billion European POC diagnostics market, growing at 11% annually, with 830,000 new colorectal and lung cancer cases per year across the EU and fewer than 40% of eligible patients currently reached by screening. Describe the commercial model: razor-and-blade, 78% gross margin on cartridges, Roche Diagnostics LOI for DACH co-distribution, phased rollout to five EU markets by Year 4. Summarise the investor validation: EUR 6.2 million raised from HTGF and Earlybird, EUR 15 million Series B from EQT Life Sciences and btov Partners contingent on EIC equity participation. Close with a direct and confident call to action: OncoSense has the technology, the clinical evidence, the team, and the market. EIC equity will unlock the Series B and the full commercialisation pathway. We invite the evaluation panel to progress this application to Step 2.

DRAFT